

Discontinuation of Prehospital Resuscitation

Standard:

Unsuccessful cardiopulmonary resuscitation (CPR) and other advanced life support ($\geq$ PL4) interventions may be discontinued prior to transport when this standard is followed as well as utilization of the [Discontinuation of Prehospital Resuscitation checklist](#).

Purpose:

The purpose of this standard is to allow for the discontinuation of prehospital resuscitation after the delivery of adequate and appropriate $\geq$ PL4 therapy.

Application:

1. For cardiac arrest with ongoing resuscitation efforts > 30 minutes:
 - a. Inclusion Criteria:
 - i. Adequate CPR has been administered.
 - ii. Airway has been successfully managed with verification of device placement. Acceptable management techniques include endotracheal intubation, BIAD, or cricothyrotomy.
 - iii. IV/IO access has been achieved.
 - iv. Rhythm-appropriate medications and defibrillations have been administered according to appropriate clinical guidelines.
 - v. Ultrasound use, when available, to determine any cardiac motion.
 - vi. Credentialed providers on scene are in agreement with the decision to cease efforts.
 - vii. If all of the above are met, then the $\geq$ PL5 provider will contact OLMC
 - viii. If you are presented with a valid DNR, see Exception Criteria below.
2. The $\geq$ PL5 provider may contact OLMC for termination of Resuscitation (TOR) within <30 minutes of resuscitation based upon patient presentation, clinical circumstances, and their clinical judgment, see [Criteria for Death or Withholding Resuscitation](#).
 - a. Providers should continue to resuscitate the following and transport after treatment sensitive interventions have been performed on scene due to a higher likelihood of resuscitation:
 - i. Cause of arrest is due to suspected hypothermia
 - ii. Sustained ROSC at any time during the resuscitation
 - iii. Persistently recurring or refractory ventricular fibrillation and/or pulseless tachycardia or any continued neurologic activity – eye-opening or motor response.
 - iv. Ultrasound, when available, determines cardiac motion.
3. When OLMC is involved in the decision to terminate, then resuscitative efforts must continue while:
 - a. The family is counseled on the patient's unchanged condition and impending discontinuation of efforts. If termination of efforts is anticipated, then Victim Services should be requested as early as possible.
 - b. The $\geq$ PL5 is requesting a TOR from OLMC.
4. Document all patient care and interactions with the patient's family, personal physician, medical examiner, law enforcement, and OLMC in the ePCR.
5. Providers should consider external factors such as the inability to secure the scene, cardiac arrests in a public place, etc. when deciding whether to terminate efforts or transport. This decision should be made in conjunction with law enforcement partners and OLMC.
6. Patients in the ambulance while resuscitation efforts are ongoing should be safely transported to the hospital. Resuscitation should not be discontinued en route, nor should it be discontinued and the patient removed from the ambulance. Very limited exceptions exist, and they are

Discontinuation of Prehospital Resuscitation

restricted to immediate life-saving management of another patient on the scene, for example, during mass casualty incidents.

7. Following pronouncement, the body comes under the jurisdiction of the Medical Examiner and should be treated according to the [Clinical Standard on Crime Scenes](#).

Exception Criteria:

If a valid out-of-hospital DNR is presented or found anytime during ongoing resuscitative attempts, the Providers/Responders may immediately stop the resuscitation efforts, and a time of TOR may be obtained from communications; see DNR and Advanced Directives for further information.